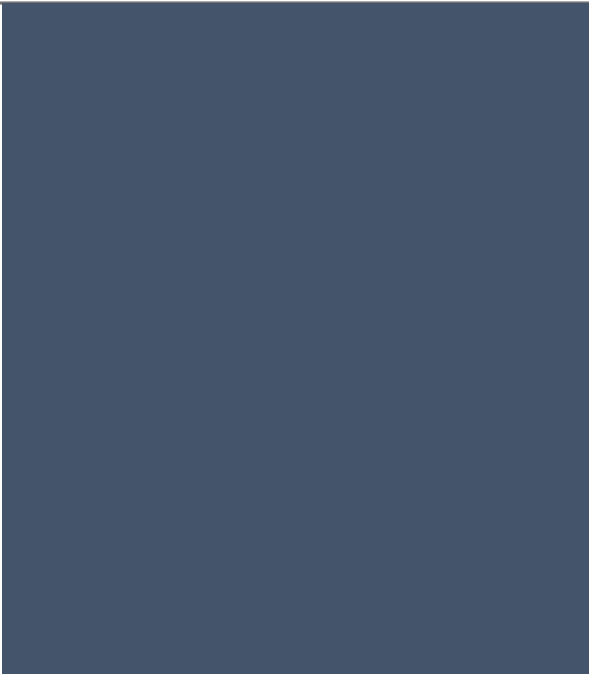




Guide For General Practise

For Adult Patients

Request

- Before prescribing drugs make a probable clinical diagnosis. Then only prescribe safe drugs.
- Never prescribe unwanted drugs.
- Always ask about the allergic history.
- If you unable to make a clinical diagnosis, send the patient to hospital.
- Never prescribe drugs: age <6months. Send them to hospital.
- Patients who are having chest pain, always go with the ECG. If the ECG shows features of ISCHEAMIA, before send the hospital, give stat doses. It will reduce the mortality rate by 60%.
- While you prescribing medications to reproductive age female always ask about the state of pregnant.
- Before nebulize the patient with breathing difficulty do the clinical examination and exclude heart failure.
- Before doing the suturing, discuss the cost of the procedure with the patient.
- Never prescribe emergency contraceptive pills and never promote abortion.
- Keep your dignity as a doctor and never keep unwanted relationships.

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01)Urinary tract infection

Ciprofloxacin 500mg bd – 3 days

or

Norfloxacin 400mg bd - 3 days

or

Nalidixic acid 600mg tds/qds- 3days

or

Nitrofurantoin 50-100mg tds 3-5 days

Acute urinary retention due to UTI

1. IV Frusemide 20mg /stat
2. Catheterization(if need)
3. Medical advices.

dysuria
urinary frequency
fever

(add pain killer{mefenamic, diclofenac,PCM})

Acute pyelonephritis – hospital admission

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02)Gastroesophageal reflux disease& gastritis

1. Omeprazole 20mg bd or 40mg bd(better)/ esomeprazole 20mg bd/ pantoprazole 40/20mg
2. Domperidone 10mg tds/metochlopramide 10mg tds(central effect)
3. Gaviscone syrup 10ml tds/belcid acid 10ml tds/ **7days**(better)

heart burn
regurgitation
dysphagia

Severe GERD & GASTRITIS

IV omeprazole 40mg /stat

IV metochlopramide 10mg/ stat

Gavicone(antacid) syrup 20ml / stat

03) Acute gastroenteritis

IF BACTERIAL

Ciprofloxacin 500mg bd

Or

Erythromycin 500mg qds/tds

Or

Cephalexin 500mg tds

Add

1. Pcm 1g tds
2. Omeprazole 20mg bd
3. Domperidone 10mg tds / 3days

Add

1. Probiotics
2. Jeewani
3. Dietary advices

IF VIRAL

probiotics **bifilac ,lactol ,protecti**

jeewani

dietary advices

pcm 1g tds

omeprazole 20mg bd

domperidone 10mg tds

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Loperamide capsule 2mg

If need add loperamide 4mg stat dose

If Severe AGE

Saline 0.9% slow infusion with IV polybion 1vial

04)Respiratory tract infections

- 1. Flu like symptoms**
- 2. Upper respiratory tract infection**
- 3. Lower respiratory tract infection**
- 4. Cough**
- 5. Asthma exacerbation**

1Flu likesymptoms

1. prednisolone 5mg tds or examethasone 0.5mg tds
2. Loratidine 10mg bd/daily or cetirizine 10mg bd/daily or desloratidine 5mg daily / bd
3. pcm 1g tds(if fever)
4. vit C 1 tab bd
5. omeprazole 20mg bd(due to predni & dexa)

If nasal congestion is there add

1. normalmsaline nasal drop 2 tds

or

2. oxymetazolin(nasivion) nasal spray 2 puffs bd

2 upper respiratory tract infection

Add antibiotic

1. amoxycilline 500mg tds

or

2. cephalixin 500mg tds

or

3. azithromycin 500mg nocte

or

4. erythromycine 500mg qds

Or

5. co-amoxyclav 625 mg tds

(this is for sore throat)

1. prednisolone 5mg tds or
examethasone 0.5mg tds2. Loratidine 10mg bd/daily
or

cetirizine 10mg bd/daily

or

desloratidine 5mg daily / bd

3. pcm 1g tds(if fever)

4. vit C 1 tab bd

5. omeprazole 20mg bd(due
to predni&dexa)**3days**

3 lower respiratory tract infection

1. prednisolone 5mg tds or
examethasone 0.5mg tds

2. Loratidine 10mg bd/daily
or
cetirizine 10mg bd/daily
or
desloratidine 5mg daily / bd

3. pcm 1g tds(if fever)

4. vit C 1 tab bd

5. omeprazole 20mg bd(due
to predni&dexa)



1. co-amoxyclav 625mg tds
or
2. clarithromycin 500mg bd
Or
3. Cephalexin 500mg tds
/ 5-7 days

IVx

Chest x-ray

FBC

SE

BU

s.cr

Always remember **CURB-65** classification If suspected pneumonia

Use antibiotics combination and supportive therapy

Eg:- **co-amoxyclav 625mg tds + clarithromycin 500mg bd for 5-7 days**(if not resolves transfer to hospital)

4. Cough

Deriphylline 75mg /150mg/300mg bd(according to the severity)

(never use deriphylline with macroloids)

Or

Ascoril syrup 10ml tds

Or

Phenycof syrup 10ml tds

Or

Benof syrup 10ml tds(expectorant)

Better :

Nebulize with normal saline 1ml+ salbutamol 1ml +/- ipravent 0.5ml
+ O₂

5. Asthma exacerbation

Moderate –

nebulize with normal saline (N/S) 1ml + salbutamol 1ml +/- ipravent 0.5ml+ O₂add prednisolone 4tabs stat

review in 20mints

if not satisfactory – nebulize again

Severe –

1. Nebulize as above. And 20 mints cycles (back to back nebulize)
2. IV hydrocortisone 200mg stat
3. if not resolves send to hospital.(for incubation & ventilation)

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Do not nebulize all patients those who have breathing difficulty.

Because some people presents with heart failure after MI of heart failure

itself (heart failure presents with bilateral ankle oedema, right hypochondrial

tenderness, weak pulse, bibasal end inspiratory crepitation).

So before you going to nebulize the patient make a clinical accurate

diagnosis.

If it is heart failure never do nebulize. It will worsen the condition.

05) Mechanical pain

1. bone pain & muscular (not arthritis)

Meloxicam 15mg bd

Or

Celecoxib 200mg bd

Or

Diclofenac sodium 50mg bd

Or

Mefenamic acid 500mg tds

Or

Tramadol 50mg tds+ domperidone 10mg tds



Omeprazole 20mg bd



**Infrared rays (IR light) +
methylsalicylate balm**

Or

Diclofenac gel

2. Arthritis (acute pain)

Aspirin 150mg

Celecoxib 200mg bd

Meloxicam 15mg bd



+ omeprazole 20mg bd

(tramadol 50mg tds + domperidone 10mg tds)

(diclofenac or ibrufen can be added)

7days or 14 days

06) Ureteric colic

Acute treatment

NBM (nil by mouth)

IV normal saline 0.9% slow infusion

Diclofenac sodium 100mg suppository / stat

IV buscopan (hyoscin bromide) 1 vial/ stat

+

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Buscopan 1 tab tds

Diclofenac sodium 50mg bd

Omeprazole 20 mg bd

Urimax(tamsolusin) 0.4 mg bd / 5-7 days

Reduction of pain is not the treatment. So then take x-ray KUB (or USS abdomen) after full bowel preparation and do needful to the patient.

08)Wounds

Burn wounds

Severe burns- transfer to hospital

Moderate to severe – transfer to hospital

Mild (superficial) - wash with normal saline
 -apply silversulfurdiazine only
 -ask not to cover with plasters
 -change dressing everyday
 -if need add antibiotics for wound infection.

Cloxacillin(500mg tds)/ flucloxacillin(500mg tds)/
erythromycin(500mg qds)/ ciprofloxacin(500mg bd),
clindamycin(300mg bd)/ co-amoxylav(625mg tds) and
pain killers add especially add proton pump inhibitors
(for pressure ulcers/ curling's ulcers)

Other wound

Arterial (isheamic) or pressure ulcers

Wound dressing with

Wash with normal saline/hypertonic
saline or betadine

Dressing with

oxoferin solution(newer drug)

Change the dressing every other day

+

Co-amoxyclav 625 mg tds 5-7 days

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Or

Clindamycine 300mg bd 5days

Or

Cloxacilline 500mg tds 5-7 days

Venous ulcers

Educate the patient (treatment options) and if possible send to teaching hospital.

Same treatment option as other wounds except wound dressing:

Betadine or

Soframycin or

Metrogel

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09) Worm treatment

1. Albendazole 400mg /stat

2. Mebendazole 200mg daily for 3 consecutive days

10)Fungal infection

1. fluconazole 50-150 mg once daily for 4-6 weeks
(before start the treatment do liver profile and confirm the hepatic damage and check liver enzymes two weekly)

2. micoral cream

3. beclomin cream

4.candid B cream

5.clotrimazole cream

According to the type of infection.

Apply clean and dry skin at least 2-3 times a day

11) Reproductive problems

1. Absence of menstruation at the reproductive age

(Amenorrhea)

Ask the LRMP

Check urine HCG

If positive send to mid wife of the area

If negative – norethisterone 5mg tds for 5-7 days

After stopping the drug menstruation
will be started. If not success, send to
VOG

2. Dysmenorrhea

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Mefenamic acid 500mg tds

Omeprazole 20mg bd

3. Change the date of menstruation.

Start norethisterone 5mg tds 1week before the
onset of menstruation and continue as you need.

Vaginal discharge

Itchy whitish colour – candidiasis – candid v3/candid v6

Vaginal tablets or

Clotrimazole cream

Watery non itchy

– azithromycin 1g stat

or

Azithromycin 1g

+ 500mg nocte for 3 days

Gonorrhea

– penicillin 500mg tds one

day or 3days

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12) Oral ulcer

Metronidazole 400mg tds (if allergy add erythromycin 500mg qds) or amoxicillin 500mg

Vit c 1 tab bd

Brufen 1 tab tds or diclofenac sodium

50mg bd + omeprazole 20 mg bd

13)Allergy

Acute treatment for

Mild

Prednisolone 8tab (40mg)

Piriton 2tab (8mg)

Loratidine(10mg) / cetirizine(10mg) /
fexofenadine(120mg) 1 tab / stat

Moderate to severe and severe

IV piriton 10mg

IV hydrocortisone 200mg

IV metochlopramide 10mg / stat

Nebulize with normal saline 1ml+salbutamol 1ml+ o₂

If angioedema+SOB (severe)

IM adrenalin 1:1000, 1ml and observe for 5mints and give
2nd dose.

+

Above treatment.

Loratidine 10mg bd or cetirizine 10mg bd or fexofenadine 120mg daily

+Prednisolone 5mg tds+ omeprazole 20mg bd

+Piridon 4mg nocte

+Hydrocortisone cream 0.1% / 3days

If the allergy is unknown use above treatment options.

It is a known allergy, avoid allergens and continue above treatments

14)Diabetes mellitus

FBS - monthly

HbA_{1c}/serum creatinine - once in three months

PPBS - monthly

Lipid profile -once in six months

Eye checkups -annually

Foot care -everyday (by themselves)

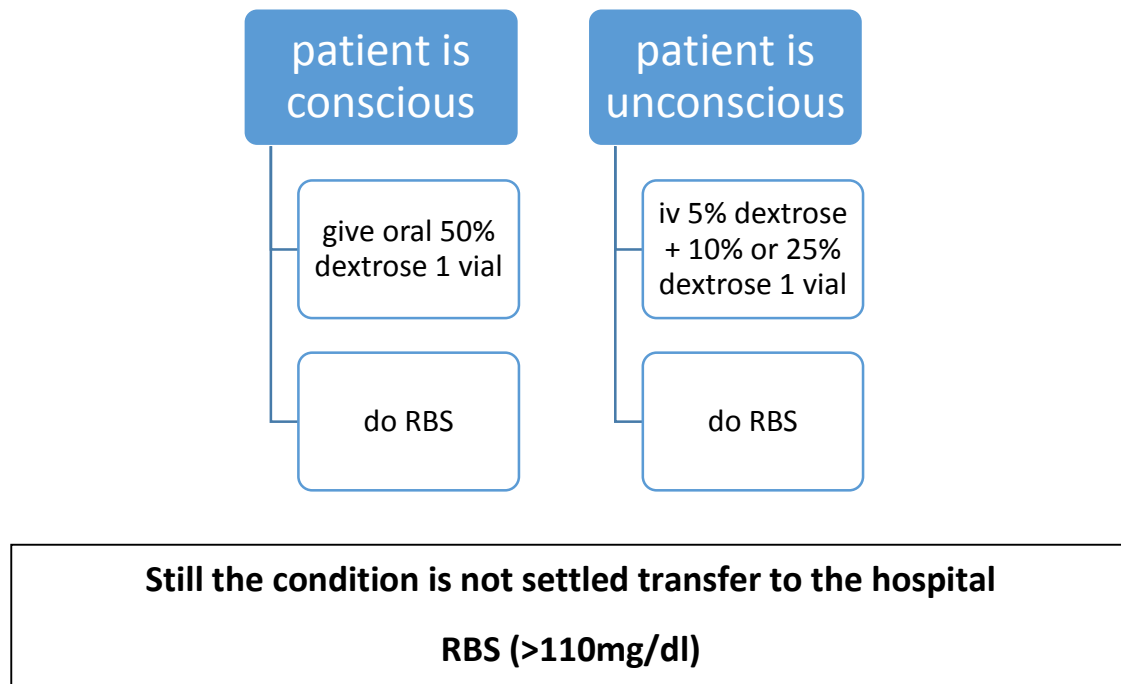
Always get the consultant opinion

1st line –metformin

2nd line –sulfonylurea/gliptins

3rd line –insulin(mixtard ,actrapid)

Hypoglycemic attack



Hyperglycemic attack

Check RBS

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1. Unreadable

Transfer to the hospital

2. >200mg/dl

Treat with oral hyperglycemics

3. >300mg/dl or more

Give short acting insulin by using a sliding scale

E.g.; - RBS – 540mg/dl

$$(540-100)/10 = \text{amount of units of insulin (IU)} = 44\text{iu}$$

Give 44units of soluble insulin (sub cut) to para umbilical area

Check RBS 45mins after the administration of insulin

Before sending home, check drugs and compliance of the patient and get the consultant opinion.

15)Hypertension

1. >200 mmHg systolic hypertension

GTN sublingual (may precipitate headache)

IV frusemide 40mg/60mg /80mg

Losartan 50mg

Check blood pressure in each 15mints interval

If it is not resolved, send to the hospital

2. >220mmHg - better to send hospital

If the patient is presented with acute chest pain, if possible do the ECG and if there are any ischemic changes send to the hospital after giving stat doses.

Aspirin 300mg (chew)

Clopidogrel 300mg

Atorvastation 40mg

GTN (sublingual)

16)Constipation

Bisacodyl 2tabs/ stat and then Bisacodyl 2tabs nocte 3days

Or

Lactulose syrup 10-20ml nocte

Or

Enema(PEG) 1 vial /stat

- High fiber diet
- Increase water intake

17)Appetite inducer

Peritol syrup (5-10ml)tds /1/12

Appeton 10ml tds

18)Otitis media

Erythromycine 500mg tds/qds

pcm 1 g tds

Or

diclofenac sodium 50mg bd

Flucloxacillin 500mf tds

+ omeprazole 20mg bd

Or

Co-amoxyclav 625mg tds

/3-5days

19) Otitis externa

Topical antibiotics

Chloramphenicol ear drops 2 drops bd

or

Ciprofloxacin 2 drops bd

Or

Gentamycin ear drops 2 drops bd

Oral antibiotics like flucloxacillin, erythromycin, co amocyclave, ciprofloxacin, metronidazole.

20) Ear wax

Waxol (NaHCO_3) 0.5% ear drops 2 drops tds – then wash with warm (partially) water in 3-4 days

21) Scabies

Benzyl benzoate 25% - apply whole over the body except around eye and keep 24 hours and wash. Do one week in consecutive days

5% permethrin – apply and keep 12 hours

Repeat in one week

22) Erectile dysfunction

Sildenafil 25-50mg /stat -½ hour before the intercourse.

(Contraindicated with heart disease)

23)Vertigo/dizziness

Check RBS , BP, pulse , and auscultate

pcm 1g tds

Stematil 1tab tds

+

piriton 4 mg nocte

Or

dexamethazone 0.5mg / prednisolone 5mg tds

Beta histidine 8-16mg bd

And treat the cause.

24)Chicken pox

Acyclovior 800mg 5 times per day / 5days

(start drugs within 1st 24hours after the onset of 1st blister appear)

25)Dandruff

ketoplus shampoo

cetrimide shampoo

26)Head lice

Lysine shampoo

Malathian shampoo

Permethrin shampoo

27)Atopic eczema

Not a curable disease. but can control.

Tacrolimus ointment

1% hydrocortisone

If Oozing – beclomin /betnovate cream

28) Acne

Doxycyclin 100mg nocte / 2 weeks

or

Clindamycine 150mg bd /7days

Or

Erythromycin 500mg tds/qds

And

Apply – benzoyl peroxide(5% or 2.5%)

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